H.R.1 HOSPITAL IMPACT MODEL · V3 · NOVEMBER 2026

H.R.1 could take $56 billion a year from U.S. hospital margins by 2032

A hospital-by-hospital scenario model of the One Big Beautiful Bill Act's Medicaid provisions, built on CMS cost reports for 4,556 hospitals and calibrated to CBO, CMS and KFF. Base scenario shown; range in brackets.

$56B
Annual margin impact in 2032 ($32–91B), 3.5% of revenue

8.0% → 4.7%
Aggregate total margin, baseline to 2032

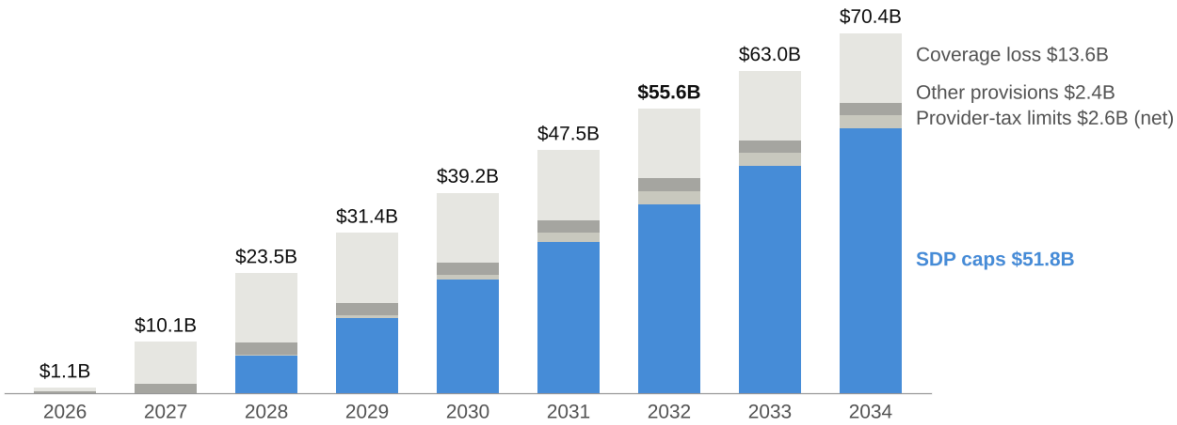
571
Profitable hospitals pushed to a loss (334–891)

$342B
Cumulative impact, 2026–2034 ($200–535B)

State-directed payment caps drive most of the hit and keep growing through 2034

SDP caps are $51.8B of the $70.4B annual impact in 2034, net of the state share hospitals finance

Base scenario, U.S. hospitals, $ billions per year (baseline-year dollars); SDP pace per CMS proposed rule



Where it hits hardest

State	2032 hit, % of revenue	Margin, baseline → 2032
Louisiana	10.8%	10.8% → 0.0%
Kentucky	10.6%	10.9% → 0.3%
New Mexico	9.8%	9.5% → −0.4%
Oregon	7.5%	0.3% → −7.7%
Arizona	7.2%	10.9% → 4.1%
Mississippi	7.1%	10.6% → 3.7%

Top Medicaid-share quartile: −8.0% of revenue, margin 5.8% → −2.5%. Lowest quartile: −0.4%. Adult Medicaid enrollment in expansion states is already down 7.8% since June 2025, before work requirements start.

What leaders can do now

1. **Stress-test by facility in Q1 2027** against your state's actual SDP preprints and tax rate. System averages hide the facilities that tip negative.
2. **Protect coverage before January 2027:** enrollment and exemption support for patients facing work requirements.
3. **Size the cost program to the gap.** A 2% cost cut, keeping a quarter of at-risk patients covered and a 0.5-point collections gain close about 70% of the 2032 gap and keep about 280 hospitals out of the red.
4. **Plan the structural cases early.** Top-Medicaid hospitals need about an 8.5% cost cut to absorb the hit, so they need state financing redesign, partnerships or service-line change, not efficiency alone.

Does the risk score work? A 2022 version of the vulnerability index, back-tested on what happened next: hospitals in its top fifth were five times as likely to close, convert or stop filing as those in the bottom fifth, and 15 of 18 Rural Emergency Hospital conversions came from its top two fifths.

Hamza Saleh, MPH · hamza.saleh.mph@outlook.com · github.com/hamzasaleh10/hr1-hospital-impact-model

Scenario estimates, not forecasts. Sources: CMS cost reports, enrollment and quality data; KFF; CBO; MACPAC; CMS 2026 proposed rules. © 2026 Hamza Saleh